

Article

Respiratory Assist Devices - Policy Article

A52517

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Contractor Information

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
CGS Administrators, LLC	DME MAC	17013 - DME MAC	J-B	Illinois
				Indiana
				Kentucky
				Michigan
				Minnesota
				Ohio
				Wisconsin
CGS Administrators, LLC	DME MAC	18003 - DME MAC	J-C	Alabama
				Arkansas
				Colorado
				Florida
				Georgia
				Louisiana
				Mississippi
				New Mexico
				North Carolina
				Oklahoma
				Puerto Rico
				South Carolina
				Tennessee
				Texas
				Virgin Islands

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
				Virginia West Virginia
Noridian Healthcare Solutions, LLC	DME MAC	16013 - DME MAC	J-A	Connecticut Delaware District of Columbia Maine Maryland Massachusetts New Hampshire New Jersey New York - Entire State Pennsylvania Rhode Island Vermont
Noridian Healthcare Solutions, LLC	DME MAC	19003 - DME MAC	J-D	Alaska American Samoa Arizona California - Entire State Guam Hawaii Idaho Iowa Kansas Missouri - Entire State Montana Nebraska Nevada North Dakota Northern Mariana Islands

6/4/26, 9:11 PM

Article - Respiratory Assist Devices - Policy Article (A52517)

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
				Oregon South Dakota Utah Washington Wyoming

Article Information

General Information

Article ID

A52517

Article Title

Respiratory Assist Devices - Policy Article

Article Type

Article

Original Effective Date

10/01/2015

Revision Effective Date

06/09/2025

Revision Ending Date

N/A

Retirement Date

N/A

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Article Guidance

Article Text

NON-MEDICAL NECESSITY COVERAGE & PAYMENT RULES

For any item to be covered by Medicare, it must 1) be eligible for a defined Medicare benefit category, 2) be reasonable and necessary for the diagnosis or treatment of illness or injury or to improve the functioning of a malformed body member, and 3) meet all other applicable Medicare statutory and regulatory requirements. Information provided in this policy article relates to determinations other than those based on Social Security Act §1862(a)(1)(A) provisions (i.e. "reasonable and necessary").

Respiratory assist devices are covered under the Durable Medical Equipment benefit (Social Security Act §1861(s)(6)). In order for a beneficiary's equipment to be eligible for reimbursement the reasonable and necessary (R&N) requirements set out in the related Local Coverage Determination must be met. In addition, there are specific statutory payment policy requirements, discussed below, that also must be met.

Accessories are separately reimbursable when used with E0470, E0471.

No aspect of a home sleep test, including but not limited to delivery and/or pickup of the device, may be performed by a DME supplier. This prohibition does not extend to the results of studies conducted by hospitals certified to do such tests or to tests conducted in facility-based sleep laboratories.

Services of a respiratory therapist are non-covered under the DME benefit.

A liner used in conjunction with a PAP mask is considered comfort/convenience item. There is no additional payment for liners used with a PAP mask. These products should be coded

A9270 (Noncovered item or service) in accordance with the Medicare Benefit Policy Manual (CMS Pub. 100-02) Chapter 15, Section 110.1.

Claims for A9279 (MONITORING FEATURE/DEVICE, STAND-ALONE OR INTEGRATED, ANY TYPE, INCLUDES ALL ACCESSORIES, COMPONENTS AND ELECTRONICS, NOT OTHERWISE CLASSIFIED) are denied as statutorily non-covered (No Medicare benefit).

REQUIREMENTS FOR SPECIFIC DMEPOS ITEMS PURSUANT TO Final Rule 1713 (84 Fed. Reg Vol 217)

Final Rule 1713 (84 Fed. Reg Vol 217) requires a face-to-face encounter and a Written Order Prior to Delivery (WOPD) for specified HCPCS codes. CMS and the DME MACs provide a list of the specified codes, which is periodically updated. The required Face-to-Face Encounter and Written Order Prior to Delivery List is available [here](#).

Claims for the specified items subject to Final Rule 1713 (84 Fed. Reg Vol 217) that do not meet the face-to-face encounter and WOPD requirements specified in the LCD-related Standard Documentation Requirements Article (A55426) will be denied as not reasonable and necessary.

If a supplier delivers an item prior to receipt of a WOPD, it will be denied as not reasonable and necessary. If the WOPD is not obtained prior to delivery, payment will not be made for that item even if a WOPD is subsequently obtained by the supplier. If a similar item is subsequently provided by an unrelated supplier who has obtained a WOPD, it will be eligible for coverage.

POLICY SPECIFIC DOCUMENTATION REQUIREMENTS

REPLACEMENT OF ACCESSORIES FOR MEDICARE-PAID, BENEFICIARY-OWNED EQUIPMENT:

For claims for replacement accessories (e.g., interfaces, tubing, filters, humidifier chambers), if Medicare paid for the base RAD initially (i.e., for 13 months of continuous use), the medical necessity for the beneficiary-owned base RAD is assumed to have been established. Therefore, to make a payment determination, there must only be documentation that the base DME item continues to meet medical need; and (2) The replacement of specific accessories or furnishing of new accessories remain medically necessary and are essential for the effective use of the base DME.

Documentation of continued medical need for the base item must come from the treating practitioner's records. The supplier's documentation records must support the need to replace the accessory to maintain the equipment's functionality and meet the beneficiary's medical need.

This guidance does not apply RADs when Medicare did not originally provide payment for the base item. In cases where Medicare did not originally pay for the DME item, all coverage, coding and documentation requirements in effect for the date of service (DOS) on the claim

under review must be met (see "Coverage Indications, Limitations, and/or Medical Necessity" section of the related LCD).

In addition to policy specific documentation requirements, there are general documentation requirements that are applicable to all DMEPOS policies. These general requirements are located in the DOCUMENTATION REQUIREMENTS section of the LCD.

Refer to the LCD-related Standard Documentation Requirements article, located at the bottom of this Policy Article under the Related Local Coverage Documents section for additional information regarding GENERAL DOCUMENTATION REQUIREMENTS and the POLICY SPECIFIC DOCUMENTATION REQUIREMENTS.

MODIFIERS

Proper use of modifiers is discussed below. Specific modifiers must be used and differ depending on whether or not the requirements outlined in the documentation section have been met.

KX MODIFIER:

Suppliers must add the KX modifier to claim lines billed for a respiratory assist device (E0470 or E0471), and related supplies and accessories, only if all of the coverage criteria in the "Coverage Indications, Limitations, and/or Medical Necessity" section in the related LCD and/or in the related NCD 240.9 have been met and evidence of such is retained in the supplier's files and available to the DME MAC upon request.

If the requirements for the KX modifier are not met, the KX modifier must not be used.

GA AND GZ MODIFIERS:

If all of the coverage criteria have not been met, the GA or GZ modifier must be added to a claim line for the RAD equipment (E0470 or E0471) and accessories. When there is an expectation of a medical necessity denial, suppliers must enter the GA modifier on the claim line if they have obtained a properly executed Advance Beneficiary Notice (ABN) or the GZ modifier if they have not obtained a valid ABN.

Claim lines billed without a GA, GZ or KX modifier will be rejected as missing information.

CODING GUIDELINES

A respiratory assist device (RAD) without backup rate (E0470) delivers adjustable, variable levels (within a single respiratory cycle) of positive air pressure by way of tubing and a noninvasive interface (such as a nasal, oral, or facial mask) to assist spontaneous respiratory efforts and supplement the volume of inspired air into the lungs. A respiratory cycle is defined as an inspiration, followed by an expiration.

A respiratory assist device (RAD) with backup rate (E0471) delivers adjustable, variable levels (within a single respiratory cycle) of positive air pressure by way of tubing and a noninvasive interface (such as a nasal or oral facial mask) to assist spontaneous respiratory efforts and supplement the volume of inspired air into the lungs. In addition, it has a timed backup feature to deliver this air pressure whenever sufficient spontaneous inspiratory efforts fail to occur.

HCPCS code A4604 describes tubing used with a heated humidifier and has a heated wire running the length of the tubing. It is designed for use with a positive airway pressure device and a non-invasive interface - i.e., nasal or face mask, nasal cannula, or oral interface.

HCPCS code A7032 is used for a replacement nasal mask interface that goes around the nose, but not into the nostrils. The unit of service for this code is “each”.

HCPCS code A7033 is used for a replacement nasal cannula-type interface. This interface extends a short distance into the nostrils. The unit of service for this code is “pair”. For some products, there are two physically separate cushions or “pillows” – one for each nostril. Two cushions/ pillows equal one unit of service of A7033. For other products, the interface is a single piece with two protrusions that extend into the nostrils. One of these interfaces equals one unit of service of A7033.

HCPCS code A7027 (Combination oral/nasal mask, used with continuous positive airway pressure device, each) is a two-piece system with separate elements for oral and nasal use.

A liner is a soft, flexible material, which is placed between the patient’s skin and the PAP mask interface. Liners used with a PAP mask are made of cloth, silicone or other materials. Liners are not interfaces for use with a PAP mask. Consequently, liners should not be billed as replacement features of a PAP mask such as A7031 (Face mask interface, replacement for full face mask, each) or A7032 (Cushion for use on nasal mask interface, replacement only, each). Liners billed as replacement features of a PAP mask should be coded A9270 (Non-covered item or service).

Monitoring devices (integrated or modular) are capable of tracking data generated by a RAD device, which can be subsequently downloaded for further analysis by a healthcare provider, DME supplier, or beneficiary. Such technologies include, but are not limited to:

- Smart cards and readers
- USB/Thumb drive accessories
- Wired telephonic transmission modules
- Wireless modems

HCPCS code A9279 (MONITORING FEATURE/DEVICE, STAND-ALONE OR INTEGRATED, ANY TYPE, INCLUDES ALL ACCESSORIES, COMPONENTS AND ELECTRONICS, NOT

OTHERWISE CLASSIFIED) describes any type of monitoring technology. Code A9279 is all-inclusive and is to be used whether the monitoring technology is incorporated as part of a base item, supplied as an add-on module or is a stand-alone item.

Use of multiple instances of A9279 to bill separately for individual monitoring features is incorrect coding.

There is no Medicare benefit or payment to DMEPOS suppliers for remote monitoring services. Suppliers must not bill A9279 for remote monitoring services

Claims billed for monitoring technologies using other NOC codes such as E1399 [DURABLE MEDICAL EQUIPMENT, MISCELLANEOUS] will be denied as incorrect coding.

Suppliers should contact the Pricing, Data Analysis and Coding (PDAC) Contractor for guidance on the correct coding of these items.

Coding Information

CPT/HCPCS Codes

N/A

ICD-10-CM Codes that Support Medical Necessity

N/A

ICD-10-CM Codes that DO NOT Support Medical Necessity

N/A

ICD-10-PCS Codes

N/A

Additional ICD-10 Information

N/A

Bill Type Codes

Contractors may specify Bill Types to help providers identify those Bill Types typically used to report this service. Absence of a Bill Type does not guarantee that the article does not apply to that Bill Type. Complete absence of all Bill Types indicates that coverage is not influenced by Bill Type and the article should be assumed to apply equally to all claims.

N/A

Revenue Codes

Contractors may specify Revenue Codes to help providers identify those Revenue Codes typically used to report this service. In most instances Revenue Codes are purely advisory. Unless specified in the article, services reported under other Revenue Codes are equally subject to this coverage determination. Complete absence of all Revenue Codes indicates that coverage is not influenced by Revenue Code and the article should be assumed to apply equally to all Revenue Codes.

N/A

Revision History Information

Revision History Date	Revision History Number	Revision History Explanation
06/09/2025	R16	Revision Effective Date: 06/09/2025 MODIFIERS: Removed: KX modifier billing instructions regarding claims for the first through third months, the fourth month, and any month thereafter

Revision History Date	Revision History Number	Revision History Explanation
		Revised: KX modifier instruction to include use when coverage criteria is met in the "Coverage Indications, Limitations, and/or Medical Necessity" section in the related LCD and/or in the related NCD 240.9 have been met and evidence of such is retained in the supplier's files and available to the DME MAC upon request Removed: "In all of the situations above describing use of the KX modifier" from the GA and GZ modifiers instructions RELATED NATIONAL COVERAGE DOCUMENTS: Added: NCD 240.9 <i>10/09/2025: At this time the 21st Century Cures Act applies to new and revised LCDs which require comment and notice. This revision is to an article that is not a local coverage determination.</i>
08/08/2021	R15	Revision Effective Date: 08/08/2021 CODING GUIDELINES: Removed: Coding and billing information pertaining to multifunction ventilators coded E0467 CODING VERIFICATION REVIEW: Removed: Section header and PDAC coding verification review (CVR) information, including CVR information pertaining to products coded E0467 <i>05/16/2024: At this time the 21st Century Cures Act applies to new and revised LCDs which require comment and notice. This revision is to an article that is not a local coverage determination.</i>
08/08/2021	R14	Revision Effective Date: 08/08/2021 REQUIREMENTS FOR SPECIFIC DMEPOS ITEMS PURSUANT TO FINAL RULE 1713 (84 FED. REG VOL 217): Removed: "The link will be located here once it is available." Added: "The required Face-to-Face Encounter and Written Order Prior to Delivery List is available here." with a hyperlink to the list

Revision History Date	Revision History Number	Revision History Explanation
		<i>04/14/2022: At this time the 21st Century Cures Act applies to new and revised LCDs which require comment and notice. This revision is to an article that is not a local coverage determination.</i>
08/08/2021	R13	Revision Effective Date: 08/08/2021 CODING GUIDELINES: Revised: Coding instructions for multifunction ventilators (E0467) (Effective 01/01/2019) CODING VERIFICATION REVIEW: Added: Section header and PDAC coding verification review (CVR) information Added: CVR requirement for products coded E0467 <i>11/11/2021: At this time the 21st Century Cures Act applies to new and revised LCDs which require comment and notice. This revision is to an article that is not a local coverage determination.</i>
08/08/2021	R12	Revision Effective Date: 08/08/2021 NON-MEDICAL NECESSITY COVERAGE AND PAYMENT RULES: Added: Language regarding no aspect of a home sleep test may be performed by a DME supplier Revised: Language regarding a liner used in conjunction with a PAP mask are noncovered Added: Language regarding monitoring devices are statutorily non-covered (previously under Coding Guidelines) POLICY SPECIFIC DOCUMENTATION REQUIREMENTS: Revised: Coverage, coding and documentation requirements reference from “see below” to “see Coverage Indications, Limitations, and/or Medical Necessity section of the related LCD”

Revision History Date	Revision History Number	Revision History Explanation
		MODIFIERS: Removed: Reference to “Group I – IV” from KX modifier section as groups are no longer referenced in the Coverage Indications, Limitations, and/or Medical Necessity section of the related LCD Revised: Typographical errors to add commas after “Limitations” when referencing “Coverage Indications, Limitations, and/or Medical Necessity” Added: Statement of claim line rejection if billed without GA, GZ or KX modifier (previously noted in the LCD) CODING GUIDELINES: Revised: Language related to HCPCS code A9279 and monitoring devices and services <i>06/24/2021: At this time 21st Century Cures Act applies to new and revised LCDs which require comment and notice. This revision is to an article that is not a local coverage determination.</i>
04/03/2020	R11	Revision Effective Date: 04/03/2020 CODING GUIDELINES: Revised: Guidance for billing HCPCS code E0467 based on DOS <i>07/16/2020: At this time 21st Century Cures Act applies to new and revised LCDs which require comment and notice. This revision is to an article that is not a local coverage determination.</i>
01/01/2020	R10	Revision Effective Date: 01/01/2020 NON-MEDICAL NECESSITY COVERAGE AND PAYMENT RULES: Removed: REQUIREMENTS FOR SPECIFIC DMEPOS

Revision History Date	Revision History Number	Revision History Explanation
		ITEMS PURSUANT TO 42 CFR 410.38(g) section REQUIREMENTS FOR SPECIFIC DMEPOS ITEMS PURSUANT TO FINAL RULE 1713 (84 Fed. Reg Vol 217): Added: Section and related information based on Final Rule 1713 MODIFIERS: Revised: “physician” to “practitioner” ICD-10 CODES THAT SUPPORT MEDICAL NECESSITY: Revised: Section header “ICD-10 Codes that are Covered” updated to “ICD-10 Codes that Support Medical Necessity” ICD-10 CODES THAT DO NOT SUPPORT MEDICAL NECESSITY: Revised: Section header “ICD-10 Codes that are Not Covered” updated to “ICD-10 Codes that DO NOT Support Medical Necessity” <i>02/27/2020: At this time 21st Century Cures Act applies to new and revised LCDs which require comment and notice. This revision is to an article that is not a local coverage determination.</i>
01/01/2019	R9	Revision Effective Date: 01/01/2019 CODING GUIDELINES: Revised: E0467 Coding Guidelines to include custom fabricated oral appliances <i>04/04/2019: At this time 21st Century Cures Act applies to new and revised LCDs which require comment and notice. This revision is to an article that is not a local coverage determination.</i>
01/01/2019	R8	Revision Effective Date: 01/01/2019 CODING GUIDELINES:

Revision History Date	Revision History Number	Revision History Explanation
		Added: E0467 Coding Guidelines <i>03/07/2019: At this time 21st Century Cures Act applies to new and revised LCDs which require comment and notice. This revision is to an article that is not a local coverage determination.</i>
01/01/2017	R7	Revision Effective Date: 01/01/2017 NON-MEDICAL NECESSITY COVERAGE & PAYMENT RULES: Added: Direction for REPLACEMENT OF ACCESSORIES FOR MEDICARE-PAID, BENEFICIARY-OWNED EQUIPMENT <i>06/14/2018: At this time 21st Century Cures Act applies to new and revised LCDs that restrict coverage, which require comment and notice. This revision is to an article that is not a local coverage determination.</i>
01/01/2017	R6	Revision Effective Date: 01/01/2017 NON-MEDICAL NECESSITY COVERAGE AND PAYMENT RULES Added: 42 CFR 410.38(g) language, previously in POLICY SPECIFIC DOCUMENTATION REQUIREMENTS section. <i>04/05/2018: At this time 21st Century Cures Act applies to new and revised LCDs that restrict coverage, which require comment and notice. This revision is to an article that is no a local coverage determination.</i>
01/01/2017	R5	Revision Effective Date: 01/01/2017 POLICY SPECIFIC DOCUMENTATION REQUIREMENTS: Added: 42 CFR 410.38(g) and Modifiers requirements

Revision History Date	Revision History Number	Revision History Explanation
RELATED LOCAL COVERAGE DOCUMENTS: Added: LCD-related Standard Documentation Requirements Language Article		
07/01/2016	R4	Effective July 1, 2016 oversight for DME MAC Articles is the responsibility of CGS Administrators, LLC 18003 and 17013 and Noridian Healthcare Solutions, LLC 19003 and 16013. No other changes have been made to the Articles.
11/05/2015	R3	Revision Effective Date: 11/05/2015 NON-MEDICAL NECESSITY COVERAGE AND PAYMENT RULES: Revised: Standard Documentation language to remove start date verbiage from Prescription Requirements
10/01/2015	R2	Revision Effective Date: 12/01/2014 (May 2015 Publication) NON-MEDICAL NECESSITY COVERAGE & PAYMENT RULES: Added: Non-coverage statement for liners used in conjunction with a PAP mask Removed: "When required by state law" from ACA new prescription requirements CODING GUIDELINES: Added: Coding guidelines for liners used with PAP mask based on DME MAC article posted on February 13, 2014 Added: Coding guidelines for Monitoring Technology based on DME MAC article posted on November 15, 2013
10/01/2015	R1	Revision Effective Date: 10/01/2015 NON-MEDICAL NECESSITY COVERAGE AND

Revision History Date	Revision History Number	Revision History Explanation
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PAYMENT RULES:
Added: ACA 6407 prescriber requirements

Associated Documents

Related Local Coverage Documents

- Articles
- [A55426 - Standard Documentation Requirements for All Claims Submitted to DME MACs](#)
- LCDs
- [L33800 - Respiratory Assist Devices](#)

Related National Coverage Documents

- NCDs
- [240.9 - Noninvasive Positive Pressure Ventilation \(NIPPV\) in the Home for the Treatment of Chronic Respiratory Failure \(CRF\) Consequent to Chronic Obstructive Pulmonary Disease \(COPD\)](#)

Statutory Requirements URLs

N/A

Rules and Regulations URLs

N/A

CMS Manual Explanations URLs

N/A

Other URLs

N/A

Public Versions

Updated On	Effective Dates	Status	
10/03/2025	06/09/2025 - N/A	Currently in Effect	You are here
05/09/2024	08/08/2021 - 06/08/2025	Superseded	View
04/07/2022	08/08/2021 - N/A	Superseded	View
11/05/2021	08/08/2021 - N/A	Superseded	View
06/18/2021	08/08/2021 - N/A	Superseded	View

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Keywords

N/A